

ONTARIO
SUPERIOR COURT OF JUSTICE

B E T W E E N:

CHRISTINE LORTIE and LAWRENCE
KEITH LORTIE

Plaintiffs

- and -

THE CORPORATION OF THE COUNTY
OF HASTINGS Operating as “QUINTE
WEST DAYCARE” and THE CITY OF
QUINTE WEST

Defendants

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) William E. Baker, Counsel for the Plaintiffs
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) Russell Palin, Counsel for the Defendants
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) **HEARD:** October 9,10,11,12,22,23,24,25,
November 5, 2007

REASONS FOR JUDGMENT

GUNSOLUS, J

[1] The plaintiffs Christine Lortie and Lawrence Keith Lortie claimed that on or about the 23rd of March, 2001 when Mrs. Lortie went to the Quinte West Daycare premises to drop off her son, Matthew Lortie, she was carefully walking along the walkway leading to the entrance to the building when suddenly without warning, she slipped and fell backwards causing severe personal injuries.

[2] The plaintiffs claimed that the cause for Mrs. Lortie’s fall was the slippery surface condition of the walkway which was composed of preformed concrete slabs, one of which had fractured and created a hollow where water and ice had gathered.

[3] The Quinte West Daycare Centre is owned and operated by the defendant The Corporation of the County of Hastings. The claim against the Corporation of the City of Quinte

West was dismissed, without costs, on consent of the parties by order of Justice Robert Scott dated the 5th of March, 2003. The OHIP subrogation claim was settled and dismissed on consent.

[4] The plaintiffs allege that Mrs. Lortie has suffered a serious injury as a result of her slip and fall and maintains a claim against the daycare centre pursuant to the provisions of the *Occupiers Liability Act*, R.S.O., 1990, CO.2.

PLAINTIFF'S WITNESSES

[5] Mrs. Lortie's evidence was that she slipped and fell on a small piece of ice that had formed in a hollow in the first concrete slab of the sidewalk leading to the daycare centre. She described this ice as being white ice approximately the size of two ice cubes. Mrs. Lortie otherwise described the sidewalk as being free and clear of ice and snow. Mrs. Lortie and her family have been attending the daycare centre since April of 1999. Mrs. Lortie gave evidence that the condition of the sidewalk had been the same for the time period that she and her family attended the daycare centre. It was acknowledged by all parties that the condition of the sidewalk has been materially the same for a considerable period of time and at least in excess of 20 years. The first slab of the cement walkway, adjacent to the municipal sidewalk, has been in a cracked condition for in excess of 20 years.

[6] The evidence of both the plaintiffs and the defendant was that there had never been a prior or subsequent complaint relative to ice or snow on the sidewalk, or as to the condition of the sidewalk itself. The plaintiffs did allege that they knew of at least one other individual who allegedly slipped on ice on the sidewalk, but no evidence was called in support of that allegation. The sidewalk was replaced in 2004. Apparently no one witnessed Mrs. Lortie's slip and fall.

[7] Mrs. Lortie gave her history both at discovery and trial. Counsel filed the transcripts of discovery as exhibits at trial. She indicated that she left High School at the age of 16 years of age during her grade ten year in 1990. She did not return to High School until 1992 when she attended High School for one semester but "dropped out" as she put it. In 1992 she worked for approximately two months. Throughout this period of time she indicated that she was periodically on various forms of Social Assistance and worked two or three months.

[8] In 1994 she moved in with her husband to be, Lawrence Lortie, and in 1996 attended Loyalist College in Belleville for upgrading. She became pregnant and did not complete the upgrading. In the subsequent two years she obtained seasonal employment at a local golf course which was briefly interrupted by a Workers Compensation claim. She then stayed home to raise her oldest son during the years 1998 and 1999 until she again attended Loyalist College in Belleville, Ontario again for upgrading purposes. It was in 1999 that she began taking her eldest son to the Quinte West Daycare Centre while she attended school. In the year 2000 she worked part-time at Zeller's in Trenton but indicated that she left that employment due to the pressures

of her new educational endeavors in a “Developmental Service Worker” course and child care of her own son.

[9] In her evidence Mrs. Lortie indicated that prior to the March 23rd, 2001 incident; she had been unable to pass two required tests in Pharmacology and perhaps tried the test a third time after her fall without success, although she was not clear on if or when the third attempt occurred. Subsequently she became pregnant with her second child and has not returned to school or looked for employment since, explaining that she is unable to do so due to the injuries that she sustained and in particular the migraine headaches that she has suffered since the March 23rd, 2001 slip and fall. She indicated that she had been extremely happy at school and in her school placements and is upset that, as a result of her injuries, her personality, as she put it, has changed. She had been happy, going to school, getting an education and “changing my life”.

[10] Mrs. Lortie’s evidence was, on the 23rd of March, 2001, as she and her son approached the daycare centre, her son was running ahead of her and she was carrying a backpack and only remembers slipping and falling backward. She said that she bashed her head, got up and saw blood on her finger and felt a goose egg on her head. She indicated that she advised daycare staff. After leaving her son at the daycare centre she was able to drive to pick up a fellow student, make a class presentation and attend her placement. She said that her neck started to hurt and she had a headache which worsened to the point that, after three or four days, the pain was so bad that she could not see. She described her neck starting to spasm and jump around and said that her tendons and muscles were standing out on her neck “like wings”. She does not recall attendances at the hospital, which occurred approximately one week following the slip and fall. She does not recall treatment at the hospital as all of that is a “blur” due to her excruciating pain.

[11] In relation to the day of the fall, Mrs. Lortie indicates that she cannot recall the exact size of the ice that she fell on but did recall that the parking lot, the public sidewalk and the walkway in question appeared to be clear, “as otherwise I would have been careful”. “There was nothing to indicate that I had to be careful.” “I would have walked around it.” The ice was “small enough that it was overlooked.” She indicated that she had on winter boots with a good tread and that she had never had a problem walking in these winter boots. Mrs. Lortie acknowledged that she had probably crossed the sidewalk in question over six hundred times commencing in April of 1999 until the 23rd of March, 2001. She further confirmed that the slab was in the same condition during this entire time period; that she never complained about its condition to the daycare centre; that she never complained about ice or water accumulating on the slab in question before the 23rd of March, 2001.

[12] Mrs. Lortie described her post injury circumstances as consisting of pounding headaches that she believes will continue for her lifetime. She described herself as having only three or four good days per month following treatments at the Rothbart Pain Clinic in Toronto. She explained that she is no longer able to enjoy a litany of activities that she formerly did, including but not limited to, baseball, golf, playing cards and walking. Mrs. Lortie further indicated that she is unable to do housework or laundry and relies heavily upon her husband, and, to a lesser

extent, her mother and sister to assist with these activities. She further indicated that her ability to play with her children is limited and that as a result of efforts in that regard she will end up in bed for days.

[13] While her days in hospital subsequent to the slip and fall remain a blur to Mrs. Lortie, she did acknowledge that she was able to leave the hospital in order to smoke and recalls “yelling and bellowing” for more pain relief and her own doctor. Nurses’ notes indicate that her mother brought in take-out food for her to eat; that she refused medication on at least one occasion; that she was able to sleep well; that she did not mention having slipped and fallen during her initial hospital stay.

[14] It is also to be noted that both plaintiffs declared bankruptcy subsequent to the commencement of this action. Both plaintiffs gave the reasons for financial difficulty as being lack of income due to Mr. Lortie’s health (See Exhibit 7, Tab 13). Notwithstanding having signed a sworn statement as to their affairs as bankrupts, both Mrs. Lortie and Mr. Lortie gave evidence at trial that those statements were in error and indicated that the reason for their bankruptcy was in relation to Mr. Lortie’s having to absent himself from work to care for Mrs. Lortie and to drive her to Toronto for treatment. Under cross-examination Mr. Lortie acknowledged that he was having other problems at work unrelated to his wife’s accident.

[15] In her evidence Mrs. Lortie also acknowledged the correctness of a letter from a Chiropractor Dr. Clements dated the 5th of May, 1997 (See Exhibit 11, page 14). In it Dr. Clements refers to the plaintiff’s disclosure to him of “pain head to toe for years....going on for years....the past year and a half has been particularly bad”.

[16] Lawrence Lortie gave evidence describing his wife’s condition post-injury. He described a tragic and dramatic change in his wife, confirming her inability to enjoy activities that she had in the past and the fact that he was now primarily responsible for housekeeping, laundry, helping their children with homework and attending at their extracurricular activities. Mr. Lortie has suffered anxiety himself, as he has felt that at times his wife has left their children unsupervised due to her illness. In reviewing his wife’s medical records he acknowledged that she did not seek medical attention until the 30th of March, 2001 subsequent to the slip and fall on the 23rd of March, 2001. He indicated that he believed her headaches started approximately three days after the fall and that the pain became so critical that he thought she was dying. He did acknowledge Dr. Clements’ 1997 report that his wife had had “pain head to toe for years”. Mr. Lortie also acknowledged the condition of the sidewalk in question as being fractured and in the same condition as when he and his family first started attending the daycare centre beginning in 1999.

[17] Casey Kelly, who was a classmate of the plaintiff, described Christine Lortie as a good student who never missed classes, was determined, enjoyed courses, was very good in placements and well liked by everyone. Ms. Kelly acknowledged that the plaintiff had driven her to school on March 23rd, 2001 immediately after the alleged slip and fall and indicated that she could see no visible injuries and that the plaintiff was able to drive, attend class and placement.

[18] The plaintiff's mother, Elizabeth Plum, confirmed Mrs. Lortie's love of school and the dramatic change in her daughter post-injury. Notwithstanding the extensive history of illnesses described in the OHIP summaries filed in relation to Mrs. Lortie, Mrs. Plum did not recall her daughter being ill "other than a case of tonsillitis".

[19] Victoria Burleigh, the sister of the plaintiff Mrs. Lortie, also described the dramatic change that has occurred to Mrs. Lortie as a result of the slip and fall and indicated that her sister had lumps on her neck and shoulders that doctors and nurses could see. No medical reports or notes filed by the plaintiff confirmed this. She also indicated the sidewalk on which her sister fell was seldom shoveled, even though both the plaintiffs' and defendants' evidence indicated otherwise. She also claimed that her sister did not get out of bed to smoke when she was in hospital, even though Mrs. Lortie acknowledged that she did and the nurses' notes filed in this matter confirm that she did. Victoria Burleigh also acknowledged the sidewalk in question was in the same condition on the 23rd of March, 2001 as it was in 1999 and that she never complained about the sidewalk to daycare staff.

[20] Dr. Stephen Rolbin is qualified as a specialist in anesthesiology and pain management. He described the plaintiff's injuries as being a whiplash, soft tissue injury, which he attributed to the fall as symptoms of headache and neck spasms did not exist before March 23rd, 2001 to the same degree post-fall which he describes now as debilitating. His evidence was that the injuries to the plaintiff were serious and permanent for which no treatment exists such that the plaintiff: a) is unable to return to the workplace, b) will require treatment the balance of her life, c) this treatment will consist primarily of botox injections with a combination of pain medication and nerve block injections. He acknowledged that treatment should be multi-disciplinary including psychiatric treatment, behavior therapy, physiotherapy and pain management such as is available through his clinic. Although he provided a plaintiff's medical report, he acknowledged that he had been providing pain management care to the plaintiff prior to and subsequent to the preparation of his report. He described his assistance to the plaintiff as being temporary, non-curative relief of pain. When confronted under cross-examination with the report of Dr. Clements in relation to the plaintiff's ongoing pain head to toe for years and the fact that Dr. Clements was not optimistic that he could assist Mrs. Lortie; the fact that she was a smoker; that she had had a prior history of a shoulder injury, insomnia and headaches; that he was not aware of other stresses in her life such as the loss of her home as a result of flooding, an unplanned pregnancy and the stress of school, he stated he would not change his diagnosis as the plaintiff's description of pre-accident activities and post-accident activities was more important as her prior condition did not prevent her from "carrying on". In summary, Dr. Rolbin indicated that "but for the slip and fall, Mrs. Lortie would not have developed the clinical pictures that are now present". He did acknowledge that another possibility for her symptoms continuing could be that the pain is exacerbated by pre-existing psychological factors, but he described the slip and fall as the triggering event.

[21] During cross-examination, Dr. Rolbin acknowledged that he recommended a neuropsychological assessment of the plaintiff and appeared not to be aware that the plaintiff had seen Dr. Macartney-Filgate, a Neurophysiologist, Dr. Bruce Stewart a Neurologist and Dr. Henry

Berry a Neuropsychiatrist and stated that their reports would not change his diagnosis. He indicated that he had not been retained to provide ongoing treatment, (even though Mrs. Lortie receives ongoing pain treatment at his clinic, he stated it is limited to pain control not healing) rather a medical legal report and that, in reference to the plaintiff Mrs. Lortie, “she was able to cope before, not now”.

[22] Ann Weiga, a registered nurse, provided a long term need and cost analysis in relation to the plaintiff. She indicated, based on doctors’ reports and particularly that of Dr. Rolbin, that she believed the plaintiff’s injuries to be permanent. It is to be noted that she did not conclude that the injuries of the plaintiff were the result of the alleged slip and fall. She provided a list of items and treatment that might help the plaintiff carry on with, and hopefully improve, her life over time to the point where she might be able to work, volunteer or add some meaning to her life. Ms. Weiga indicated that her task is to determine this and not the costs of each item. She further advised that she based her report on materials presented to her by way of medical reports and self-reporting by both Christine Lortie and Lawrence Keith Lortie. She was not aware of Dr. Clements’ report (Exhibit 11, Tab 1, page 14) and the fact that Mrs. Lortie had complained to him of pain “head to toe” at the age of 22 of some four years duration, worsening in the one and a half years prior to her turning 22. She further was unaware that Dr. Clements indicated that he had little hope that he could provide long term benefit by way of treatment. Ms. Weiga’s report can best be described as “a wish list”.

[23] Catherine Pollack from Manufacturer’s Life sets out its subrogated claim as follows: \$15,197.64 for Botox treatments, \$32,188.94 for medications, \$1,340.00 for physiotherapy and \$580.00 for a tens machine. She acknowledged that a subrogation agreement had not been entered into between the plaintiffs and Manufacturer’s Life as required by the terms of the agreement between Mr. Lortie’s employer and Manufacturer’s Life. Deborah Carter, an economist, provided a written report filed as Exhibit 19. She gave evidence that Mrs. Lortie’s future loss of earnings would be \$750,615.00; future loss of value on non-wage benefits to age 65 \$118,496.00 for a total of \$877,111.00 and the cost of future care set out in Exhibit 19 as \$1,243,029.00. The report set loss of Canada Pension plan benefits at \$56,554.00 and future loss of ability to do housework at \$157,063.00.

DEFENCE WITNESSES

[24] Christopher Angelo was the Manager of Public Works for the City of Quinte West on the 23rd of March, 2001. He gave evidence that no maintenance was required on that day in relation to the public sidewalk adjoining the walkway where the plaintiff claims she fell. He indicated the weather conditions were such that no plowing, salting or sanding was required. The record filed in relation to the company which maintained the public parking lot where Mrs. Lortie would have parked on the day in question also confirmed that no maintenance of any nature was required on the 23rd of March, 2001.

[25] Donna McAlpine, the current supervisor of the Quinte West Daycare Centre, gave evidence in relation to the County of Hastings' unwritten verbal protocol requiring all employees to be responsible for the safety of everyone, including maintenance of the walkway in question. She indicated that the protocol was established by Hastings County and was in place when she commenced employment in 1982 and was in place at the time of the incident and applicable to the Quinte West Daycare Centre.

[26] Rhonda Garrett has been employed with the Quinte West Daycare Centre since 1998. She gave evidence that no individual is in charge of maintenance of the sidewalk in question and that no written check list was in place ensuring that sidewalk maintenance was undertaken. She did confirm the verbal protocol making all employees responsible for ensuring that the sidewalk in question was safe and that each employee undertook this obligation as they entered and exited the daycare centre. Her evidence was that since 1998, she was not aware of any other complaints of a slip and fall on the sidewalk in question and that no one ever complained in relation to the condition of the sidewalk itself. She further indicated that at no time did she ever see an accumulation of ice in the centre of the cracked patio slab and if she had, she would have immediately salted the area in question. She further gave evidence that had this been an ongoing problem, she would have advised the Health and Safety officer. In describing the sidewalk she indicated that she saw nothing wrong with it, although it was cracked, "it seemed flat."

[27] Donna Cummings was (and continues to be) an early childhood educator at the daycare centre in March of 2001. The court was extremely impressed with her evidence, demeanor and her clear desire to be accurate in her answers when giving evidence in-chief and under cross-examination. She confirmed the protocol obligation of the daycare centre staff to maintain the sidewalk. She explained that the first employee in would be required to clear the sidewalk of snow and ice and salt as required. This maintenance could occur all day long, dependant upon weather conditions. She indicated that a shovel, broom and salt were available at all times. On the 23rd of March 2001, the day the plaintiff, Christine Lortie slipped and fell, she commenced work at 7:30 a.m. She arrived for work at approximately 7:15 a.m. that morning, walking from her home to the daycare centre. On arrival, she had no difficulties and she did not observe any snow or ice on the walkway in question and, as was her usual routine, she inspected the walkway as she walked across it. She gave evidence that she does not ever recall water accumulating on the cracked patio slab. She indicated that water ran off the walkway, as it was sloped and did not recall any pooling of water. On the 23rd of March 2001 she does not recall any precipitation and remembers the day as being clear. When asked specifically whether or not she was aware of water or ice accumulating on the cracked slab, she was emphatic that none did. She confirmed that she did not hear of, nor have any complaints directed to her, in relation to the state of repair of the sidewalk or in relation to the accumulation of ice or snow. That remained true to the date when the sidewalk was replaced in 2004. Ms. Cummings gave evidence that she was so concerned as to the safety of students and parents entering the daycare centre that she would even undertake maintenance of the public sidewalk adjacent to the walkway in question. She did recall Mrs. Lortie attending the daycare centre on the 23rd of March, 2001, walking by her but not advising her of any slip and fall.

[28] The next witness to give evidence on behalf of the defendant was Deborah Sacharuk. She, too, confirmed the maintenance safety protocol in relation to the sidewalk in question indicating that it was the same policy that had been in place in the County of Hastings since 1974. She was an early childhood educator on site on the 23rd of March, 2001. She indicated that she arrived at 6:15 a.m. and was the first employee to arrive on the 23rd of March, 2001. Her evidence was that, as she crossed the walkway in question, she checked it. She confirmed that nothing needed to be done by way of maintenance on the walkway and, therefore, she proceeded to enter the daycare centre and open it for business. She gave evidence that the sidewalk in question was in the same state of repair for over 33 years. She stated that subsequent to her arrival at 6:15 a.m., the next employee would arrive at 7:30 a.m. and each employee arriving thereafter is required to, and does, undertake an inspection of the walkway to determine whether it needed to be shoveled, swept or salted. She also confirmed that before March 23, 2001 she had never heard of or received a complaint in relation to the condition of the sidewalk or in relation to the accumulation of ice or snow. She confirmed that Mrs. Lortie told her on March 23rd, 2001 that she had fallen, but did not say where she had fallen. Mrs. Lortie did not indicate to her that she had been injured. Mrs. Sacharuk did not observe any injury in relation to Mrs. Lortie. Approximately ten minutes after Mrs. Lortie left the daycare site, Deborah Sacharuk said she went outside to see where Mrs. Lortie might have fallen and to re-check the walkway in question. Her evidence was that she could see nothing on the walkway and that both the adjoining public sidewalk and the public parking area were clear of ice and snow. She did indicate that there were small patches of ice or slush on the lawn area beside the walkway. Mrs. Sacharuk provided an incident report approximately two weeks after the 23rd of March, 2001. (See Exhibit 36)

[29] Dr. Henry Berry a Neurologist, Neuropsychiatrist and Pain Assessor gave evidence on behalf of the defendant. He described himself as a specialist in assessing patients with pain who have no demonstrable basis for it. He noted in the plaintiff's medical history past episodes of headache and pain. He opined that the onset of the plaintiff's headaches and neck pain did not result from the slip and fall but rather the lumbar puncture she underwent a number of days post-incident.

[30] Indices described by Dr. Berry were as follows:

- (a) Nurses' notes describing beligerence and the refusal to cooperate by the plaintiff Mrs. Lortie.
- (b) Mrs. Lortie going from hysterical weeping to being dry eyed and swearing.
- (c) A psychotherapist noting 4 out of 8 "Waddels" signs showing an exaggerated response to touch by Mrs. Lortie.

[31] Dr. Berry concluded that Mrs. Lortie's pain was not based on physical disease but rather having an emotional base. He reviewed Mrs. Lortie's description that muscles were hard as a rock and seized up in her shoulder/neck area which he indicates happens with anxiety. Dr. Berry

reviewed a number of anxiety producers in Mrs. Lortie's life including the unplanned birth of her second child which she responded to as "no way I could take care of another child". He also related her description of the onset of nightmares some six months after the accident which did not relate to the accident but rather, of "people chasing her" and "having to protect her children". He, therefore, felt that he had to look to other life stresses as reported by the plaintiff producing:

- (a) Fatigue.
- (b) Neck pain and backaches.
- (c) Dimished memory.
- (d) The act of reading and use of a personal computer bringing on headaches.

[32] Dr. Berry went on to describe a large volume of medical records showing longstanding medical health problems extending from the plaintiff's childhood, which he concluded demonstrated a longstanding excessive illness behavior by Mrs. Lortie. (Mrs. Lortie's mother said she did not recall such a history, rather only one case of tonsillitis.)

[33] To quote from Dr. Berry's report dated the 9th of February, 2004 (See Exhibit 20, Tab 4 commencing at page 19) :

The history of progressive, daily, generalized head pain, worsened by any activities is highly unusual; it is not a feature of tension headache nor of any of the known forms of headache and is more in keeping with pain of a psychogenic origin.

Her pre-accident medical records are extensive and reveal much utilization of the healthcare system and frequent attendance at hospital Emergency and Outpatient Departments. A number of factors appear to have contributed to her poor health over the years. Her pre-accident health problems included an incomplete abortion in 1998, a fall with head injury, scalp laceration and shoulder injury in August of that year, left shoulder pain of several years' duration such that, she underwent excision...and she also was troubled with abdominal pain, gynecological difficulties, throat, chest and ear infections. The fall of March 23, 2001 did not result in any significant or permanent physical injury but it did create a focus for symptoms and for misattribution.

[34] He then went on to describe her underlying personality and historical emotional reactions noted during her hospital admission of March 31, 2001. In clinical practice, he indicated that it is recognized that some patients will develop acute and seemingly severe symptoms in relation to emotions and life stresses and this appears to have occurred here. He concluded:

In clinical practice, it is recognized that some patients will fall into the illness role as a means of adjustment to underlying emotions, expectations and the stresses inherent and life adjustment; this can occur *de novo* or after an incident or accident for which someone else is considered to be responsible, and I believe that

this has happened here. Her condition represents a type of illness behaviour with prominent somatoform features, significant pre-accident health problems and the onset of new symptoms, misattribution, and their progressive worsening during her adult years. A fall and simple blow to the head is not a medically recognized cause of the delayed onset of headaches of a constant, progressive and severely disabling type.

[35] He went on to indicate that if this slip and fall had not happened, some other stress or disappointment in life could have resulted in the same pain. Exhibit 10, page 11 is a Discharge Summary dictated and authenticated by Dr. K.J. Reynolds, Mrs. Lortie's physician. He concluded that no objective neurological signs were found. That x-rays of the skull and neck were unremarkable and confirmed at page 16 of exhibit 10, that Dr. Arcand obtained a history from Mrs. Lortie that she had suffered similar headaches some five years previously.

[36] At page 20 of Exhibit 10, Dr. Koshy's note of the 31st of March, 2001 states that Mrs. Lortie had not noted any injury or trauma to the neck and concluded that he was not able to definitely pinpoint the exact cause of her headache. On the 2nd of April, 2001 it is noted that Dr. Koshy contacted the Trenton Memorial Hospital and advised that the x-ray of Mrs. Lortie's cervical spine was normal.

FINDINGS

[37] The court does not accept the evidence of Mrs. Lortie that she fell on the walkway leading to the daycare centre or that water and ice had accumulated on the walkway on March 23rd, 2001. A more plausible explanation is that Mrs. Lortie, hurrying to keep up with her son who was, as she acknowledged, ahead of her, cut across the lawn area and slipped and fell on the ice or slush that had accumulated on the lawn area.

[38] It was the evidence of daycare staff that they did not allow snowbanks to accumulate on each side of the walkway in question, but rather fanned it out because snowbanks attract children. No evidence was presented that water or ice accumulated on the cracked slab on the day in question or at any time before or after the alleged slip and fall. Ice or slush had accumulated on the lawn area beside the walkway as shown on exhibit number 5, picture number 15.

[39] The plaintiffs provided an engineering report from Giffin Koerth dated the 27th of June, 2007 exhibit 14. Of note in this report is the observation that Mrs. Lortie's guess as to the size of the ice patch (two ice cubes) was insufficient for an assessment as to whether or not a slip was possible. Further the report acknowledges that given the lack of precipitation on that day and the temperature range in the 12 hours prior to Mrs. Lortie's fall, Giffin Koerth would not expect ice to have formed on a properly graded surface.

[40] The defendant provided two reports by Jenish Engineering Limited marked as exhibits 15(a) and 15(b) and dated the 31st of May, 2005. The first report summarized their investigation which included the following:

- (i) Examination of the accident sight on February 11th, 2005.
- (ii) Examination of one of the shoes worn by Mrs. Lortie on the 23rd of March, 2001.
- (iii) Review of the transcript at the examination for discovery of Mrs. Lortie, dated June 11th, 2002.
- (iv) Review of the monthly meteorological summary from March, 2001.
- (v) Examination of color photographs of the accident site taken shortly after the date of the accident by Lawrence Lortie.
- (vi) Examination of color photographs of the shoes worn by the plaintiff.

[41] The boots in question were a medium density rubber type material with a “winter” light tread pattern. Friction testing indicated these winter boots provided Mrs. Lortie with relatively good slip resistance on an ice surface under both wet and dry conditions. The report summarized the meteorological conditions for the month of March 2001, indicating that temperatures were above zero and that there had been no precipitation in the seven days preceding the alleged incident and, therefore, minimal ice build up would be expected. The report indicated that it would appear that the walkway in question provided sufficient drainage and that perhaps even the cracks themselves could have provided an avenue for drainage, assuming that the cracks were not filled with impervious soil or other debris.

[42] The photographs taken by Mrs. Lorie’s husband indicated that there was still some snow present in the area surrounding the subject walkway at the time he took the photographs a few days after the 23rd of March, 2001. Given the absence of any water or ice present in the centre of the broken slab, the engineers opined that there was sufficient drainage to minimize any accumulation of water and ice within the broken slab. The report went on to review the dynamics of the fall described by Mrs. Lortie, which Jenish found were not consistent with the small size of the ice patch described by Mrs. Lortie. The manner in which she described her fall indicated that her feet slipped out from under her and her body rotated rearward. The report concluded that this type of slipping described by Mrs. Lortie was consistent with a much larger ice patch. In addition Mrs. Lortie describes striking her head on the walkway on which she slipped and not on the municipal sidewalk from which she entered the walkway. The described location of the ice patch, near the middle of the cracked slab, was a very short distance from the municipal sidewalk, probably less than one metre. If Mrs. Lortie did indeed slip on the first slab and fall backwards, this projectory would place her upper body and head on the municipal

sidewalk. As such, the report concluded that Mrs. Lortie's description of the location where she struck her head was inconsistent with the described location of the ice patch.

[43] The report concludes:

1. Photographs of the walkway taken a short time after the incident indicated that the first concrete slab was cracked through the middle, along its length and width, with a small void at the point where the cracks converged near the centre of the slab.
2. Given the absence of any water or ice present in the centre of the broken slab, in the supplied photographs, this would suggest that there was sufficient drainage to minimize the accumulation of water and ice within the broken slab.
3. The small void near the centre of the cracked slab appears to match the approximate size and location of the ice patch as described by Ms. Lortie.
4. The nature of the fall described by Ms. Lortie was consistent with her feet slipping out from beneath her, causing her to fall backwards and strike her head on the sidewalk.
5. The dynamics of the fall described by Ms. Lortie were not consistent with the small size described for the ice patch.
6. The type of slipping described by Ms. Lortie was consistent with a much larger ice patch, allowing both feet to slip out from underneath her.
7. Assuming that the very corner of Ms. Lortie's heel struck the described ice patch, her foot would not likely have slid more than about five centimetres, about half the amount required to initiate a fall.
8. The dynamics of the fall described by Ms. Lortie were not consistent with the very small size of the ice patch described by Ms. Lortie, and an ice patch that size would not cause the foot to slip a sufficient distance to result in a fall.

[44] In addition to this report, Jenish Engineering Limited provided a "slip and fall" reconstruction report walkway analysis. Based on an analysis of the walkway described by Mrs. Lortie and as photographed by Mr. Lortie, on which this slip and fall incident allegedly occurred, the report concluded:

1. Assuming a slope of about 10 percent (1 in 10) as measured on the new walkway, the slope of the original walkway was not excessive, when compared with the requirements in Section 9.8.6.2 of the OBC. (*The Ontario Building Code*)

2. The original walkway did not contravene any requirements within the OBC and provided a safe design.
3. The photographs indicated that the slab closest to the municipal sidewalk was cracked and in need of replacement.

[45] The defendants also provided a report from Dr. James J. Dowling, an Associate Professor of Biomechanics, McMaster University, which report was dated the 6th of November, 2006 and filed as Exhibit 15(d). Dr. Dowling reviewed Mrs. Lortie's description of her fall from her transcript of discoveries, pages 54 to 58, and the Jenish Engineering Limited reports. Dr. Dowling concluded:

I find the plaintiff's testimony concerning the nature of her slip and fall to be inconsistent with her testimony of the conditions of the ice patch and her walking. It seems more plausible to me that the ice patch was considerably larger and that she was either running or there were other aspects perturbations that would cause a head impact from a backward fall.

[46] Jenish Engineering also provided a review of the plaintiff's engineering report filed as exhibit 15(c). Jenish Engineering pointed out that Mrs. Lortie's description of the ice patch was simply not consistent with a slip and fall.

[47] Of note, too, is the evidence of Mr. Lortie in responding to cross-examination by counsel for the defendant, Hastings County. His evidence on the 10th of October, 2007 is most telling:

We were sitting in the room. Dr. Reynolds wanted a history of what's happened in the last couple of weeks. I mentioned that she had a slip and fall. My wife was belligerent and swearing and saying, "no, no, no. We can't say that. It wasn't them. We're not scum. I ain't saying that. I'm not putting the blame on them." Then her mother spoke up and said, "No, you told me on Saturday that you had a stiff neck." I went to work. I explained what happened, because he said it was extreme whiplash, to a friend at work, and he informed me that I should call a lawyer."

LAW

[48] As stated above, I do not accept Mrs. Lortie's evidence in relation to her explanation as to the cause or location of her fall. Further, considering the weather, the time of year, the inspection and maintenance program practiced by the defendants, I am unable to find any liability on the part of the defendant. In the circumstances of a case of this nature, the presence of a small patch of ice does not amount to a breach by the defendants of the duty set out in the

Occupiers Liability Act. The defendants are not required to meet a standard of perfection as that is not what Section 3(1) of the *Occupiers Liability Act* contemplates. (See *Przelski v. Ontario Casino Corporation* [2001] O.J. No. 3012).

[49] As set out above, I do not find that Mrs. Lortie slipped and fell on the walkway of the defendant. Even if I found that the plaintiff had slipped and fallen on the walkway of the defendant, a consideration of the applicable law in Ontario would not visit liability upon the defendants.

[34] Section 3 (1) of the *Occupiers Liability Act* provides that,

An occupier of premises owes a duty to take such care as in all the circumstances of the case is reasonable to see that persons entering on the premises, and the property brought on the premises by those persons are reasonably safe while on the premises.

[35] In *Garofalo v. Canada Safeway Ltd.*, [1998] O.J. No. 302, Kozak J reviewed the case law which interpreted the above section of the *Occupiers Liability Act*. The following paragraphs taken from Kozak, J's reasons set out the principles which must guide the court in this case.

¶19. In the case of *Waldick v. Malcolm et al*, 70 O.R. (2d) 717 Ontario Court of Appeal, it is stated at page 723:

All Courts have agreed that the section imposes on occupiers an affirmative duty to make the premises reasonably safe for persons entering them, by taking reasonable care to protect such persons from foreseeable harm. The section assimilates occupier's liability with the modern law of negligence. The duty is not absolute and occupiers are not insurers liable for any damages suffered by persons entering their premises. Their responsibility is only to take such care as in all circumstances of the case is reasonable.

¶20 The Supreme Court of Canada in *Waldick* (1991) 83 D.L.R. (4th) at page 18 stated that:

The goals of the Act are to promote and indeed require where circumstances warrant, positive action on the part of occupiers to make their premises reasonably safe.

¶22 The factors which are relevant to an assessment of what constitutes reasonable care will necessarily be very specific to each fact situation...thus the proviso "such care as in all the circumstances is reasonable". (*Waldick Supra* p. 13)

¶27 At page 8 of Hoffert Bouck J goes on to state:

The plaintiff must prove the conduct of Westfair was unreasonable in the circumstances on a balance of probabilities. If I were to hold the defendant acted unreasonably in these circumstances, that would mean it would have to schedule the cleaning of the grape table area more often than every twenty-five minutes the store was open. But even that might not be sufficient to prevent harm. The law does not demand a system of constant surveillance and instant response. (*Beaman v. Canada Safeway Ltd.* (1993) Sask. Q.B. November 12, 1993 No. 3377 Lawton, J.)

¶28 The duty of care does not extend so far as to require the defendant to remove every possibility of danger. The test is one of reasonableness and not perfection. (*Carlson v. Canada Safeway Limited* 47 B.C.L.R. 252 B.C.C.A.; *Gerak v. R, in Right of B.C.* reflex, (1984) 59 B.C.L.R. 273 (C/A).

¶29 Tort law involves the attribution of the risk or the allocation of the responsibility for the accident. In this case we are dealing with a statutory duty of care imposed by Section 3 of the *Occupiers Liability Act* which requires that an occupier take such care as in all the circumstances of the case is reasonable to see that persons entering on the premises are reasonably safe when on the premises. At the same time, a person coming onto the property is expected to be reasonably alert to any obvious dangers (i.e. not be contributorily negligent) and not voluntarily assume the risk.

¶30 The onus is on the plaintiff to prove on a balance of probabilities that the defendant was in breach of a positive duty of care. She must be able to point to some act or omission on the part of the defendant which resulted in her injury. In this case I take it that she relies upon the failure of the defendant to pick up a piece of plum which was apparently discarded in front of the bin in photo B as being the cause of her injury.

¶31 The positive or affirmative duty that is imposed upon the defendant does not extend to the removal of every possible danger. It does not require the defendant to maintain a constant surveillance or lookout for potential danger. The defendant meets its duty to take reasonable care if it takes measures that are reasonable in the circumstances.

[50] In summary, the plaintiffs have not proven, on a balance of probabilities, that the defendant was in breach of a positive duty of care. They were unable to point to some act or omission on the part of the defendant which resulted in her injury. I find that the more plausible explanation for the plaintiff's injuries is that she cut across the grassed area next to the defendant's walkway and slipped and fell on ice or slush. The evidence is not clear, however,

the area where Mrs. Lortie probably fell may be owned by the defendant, City of Quinte West against whom this action was dismissed, on consent of the parties. Any action against the City of Quinte West, counsel for the defendant County of Hastings points out, was statute barred when this action was commenced.

DAMAGES

[51] As a result of her fall, the plaintiff received injuries to the back of her head and neck. Her doctors describe it as a soft tissue whiplash injury. She has suffered considerable pain from those injuries, including debilitating headaches, which she indicates she still experiences. She has only a few good days once a month where she is relatively pain free as a result of pain management treatment that she receives from Dr. Rolbin.

[52] Dr. Rolbin reports that Mrs. Lortie has had headaches only since the accident of March 23rd, 2001. Her headache is associated with severe neck pain which radiates down her left arm and forearm one or two times per week. The pains are made worse by any movements. In particular, the activities of home cleaning and child care will aggravate her pain and neck pains. The only relief she obtains from her pains are by injection of local anesthetics, as done in her local pain clinic, or by medication therapy. She never had any problems with headaches or radiculopathy prior to the accident of March 23rd, 2001.

[53] Dr. Rolbin's diagnosis is as follows:

1. **Cervicogenic headache, neck pain and shoulder pain:** As defined in the *Task Force for Taxonomy of the International Association for the Study of Pain*, this patient meets the criteria for the diagnosis of cervicogenic headache in that there is severe headache associated with shoulder and arm pain. This is provoked by mechanical mechanisms such as movement of her head. Her neck also has a reduced range of motion. The headaches are also relieved by anaesthetic blockage. Such injuries are often associated with severe neck injury which occurs in incidents such as the injury which she sustained on March 23, 2001.

2. **Sever biomechanical disorder of the cervical spine with:**

- a) cervical sprain;
- b) lumbar sprain,

These injuries are often precipitated by events involving spinal acceleration/deceleration injuries of the head and neck with respect to the trunk, as was sustained by Mrs. Lortie on March 23, 2001. Her current symptoms and physical signs present on her current exam meet the definition for these diagnoses as established by the *Task Force for Taxonomy of the International Association for the Study of Pain*.

3. Post-traumatic stress syndrome: This is in accordance with the criteria established by Sadwin, Young and Packard and Saper. She tested positive for 32 out of 40 symptoms of post-traumatic stress disorder. This diagnosis is made because of her problems with:

- a) headache;
- b) memory and concentration problems;
- c) mood changes and irritability;
- d) anxiety;
- e) sleep problems;
- d) dizziness and balance problems.

Because of the nature of her injuries, her depression may also be due to a pain disorder with psychological factors. A general medical condition such as that due to the various physical injuries which she sustained in the accident of March 23, 2001 would explain her medical condition. It is not uncommon, unfortunately, that the pain symptoms after such an accident become chronic and a psychological problem develops. It is also possible that the primary diagnosis might be an adjustment disorder with mixed anxiety and depression. These diagnoses should be further assessed by a psychologist. In either case, her symptoms are severe.

4. Radicular symptoms in her left arm and in both legs. There was also a positive Lasegue's sign present in her right lower limb. These symptoms are often associated with discogenic or zygapophyseal joint pain, which may have been sustained in the injury of this patient.

PROGNOSIS:

It has been two years and three months since the accident of March 23, 2001. Because of the impairments and the restrictions that they impose on her, it is my opinion that Mrs. Lortie is impaired at the present time with regards to domestic activity, her work and her enjoyment of life.

I believe that she will remain so for the remainder of her life. Considering the long duration since the accident, I do not believe it is realistic to expect significant improvement of her injuries with time that would enable her to increase her functional abilities.

It is probable that over time as she ages, her musculoskeletal injuries will get worse. As well her psychological problems may further deteriorate, particularly as she comes to realize that her symptoms are permanent and the impairments which she has developed will not improve.

I believe that her employability will be impossible at any time in the future. Her severe pain and other symptoms which are present on a day-to-day basis are of sufficient magnitude that she will have difficulty with her activities of daily living as well as with employability.

FURTHER ASSESSMENT AND TREATMENT:

Any recommendations offered are provided as guidance and not as medical orders. Please send a copy of my report to Mrs. Lortie's personal treating physician. I would be willing, with your permission, to discuss my opinions with her treating physician.

1. She should continue to undergo treatment with a physician expert in pain management. This may involve continuing palliative blocks, as well as additional therapy with Botox. Botox therapy also palliative in nature. Her initial treatment has not been very successful, but often with repeat doses, there is a cumulative effect. I believe it would be worthwhile to repeat with at least two further treatments. These are not covered by OHIP at the present time.
2. She should undergo psychological assessment and therapy to assist with her pain management and to optimize her psychological symptoms. I believe that psychotherapy, behaviour therapy and possibly cognitive therapy may assist her with managing her pain and lifestyle issues. At this late date, I do not believe that she will improve her impairments with these therapies, but I believe they will assist her in coping with her limitations and perhaps limit further deterioration in her symptoms and impairments.
3. Neuropsychological assessment by a neuropsychologist would further detail her cognitive impairments should this information be required.
4. Referral to a neurologist or ENT surgeon for assessment of her balance problems.
5. Should this be required, a functional abilities evaluation and functional capacity evaluation will be useful in assessing her impairment levels and her possible future employability."

[54] Dr. Rolbin then concludes:

I have been asked to comment as to whether this patient has reached the threshold of permanent, serious impairment of an important physical, mental or psychological function. I conclude that she has achieved such a threshold of

important physical, mental and psychological impairment. Significant aspects of her daily living have been compromised. She suffers major symptomatology and is unlikely of ever being able capable of returning to her previous state. I do not believe that it is realistic to expect any recovery of her injuries and with time, it is probable that she will have further deterioration. I do not believe she will be employable again. As well, should she deteriorate significantly, it is possible that she may need assistance with managing her personal and financial affairs. These problems will persist and require much on-going care for the remainder of her life.

The patient continues to suffer and to have specific limitations in her physical ability that cause exacerbation of her symptoms. I believe that these injuries are all secondary to the accident of March 23, 2001. None of her symptoms or complaints other than her depression were present prior to this accident. I believe that her current depression is related to the accident March 23, 2001.”

[55] Mrs. Lortie described herself approximately one week after the fall as being in pain to the point where she was unable to see and to the point where her stay in hospital was a blur the particulars of which she cannot recall.

[56] Prior to the accident Mrs. Lortie had a long history of health problems, as confirmed by Dr. Henry Berry, which I have already outlined above. In summary, the plaintiff is a woman who has suffered chronic pain. In *Athey v. Leonati* [1996] 3 S.C.R. 498. The Supreme Court of Canada stated the following in respect of compensation of tort law:

¶ 32 To understand these cases, and to see why they are not applicable to the present situation, one need only consider first principles. The essential purpose and most basic principle of tort law is that the plaintiff must be placed in the position he or she would have been in absent the defendant’s negligence (the “original position”). However, the plaintiff is not to be placed in a position better than his or her original one. It is therefore necessary not only to determine the plaintiff’s position after the tort but also to assess what the “original position” would have been. It is the difference between these positions, the “original position” and the “injured position”, which is the plaintiff’s loss.....

[57] Keeping in mind the *Athey* case, I assess the plaintiff’s damages for pain and suffering and loss of enjoyment of life at \$60,000.00.

[58] I would have also allowed the subrogated claim of Manulife in the amount of \$49,306.58.

[59] In respect of the *FLA* claim by the plaintiff Lawrence Lortie, I assess the amount of \$15,000.00. I note that an *FLA* claim was not plead on behalf of the Lorties two children.

[60] As to the claims for future economic loss and the cost of future care, without more evidence relevant to Ms. Lortie's specific circumstances, the court is unable to assess what these damages would otherwise have been.

[61] In summary, I assess the plaintiff's damages at \$60,000.00, Manufacturer's Life subrogated claim at \$49,306.58 and Mr. Lortie's *FLA* claim at \$15,000.00. The plaintiff's action, however, is dismissed.

COSTS

[62] If counsel are not able to agree as to costs, the defendants may make written submissions of no more than three pages, including a schedule of costs, by January 24, 2008. The plaintiffs may then make written submissions of no more than three pages by February 4, 2008, and the defendants may submit a brief reply, if necessary, by February 11, 2008.

"Gunsolus, J"

Gunsolus, J

Released: January 14, 2008

COURT FILE NO.: 004756 (Cobourg)

ONTARIO

SUPERIOR COURT OF JUSTICE

B E T W E E N:

CRISTINE LORTIE and LAWRENCE KEITH
LORTIE

Plaintiffs

- and -

THE CORPORATION OF THE COUNTY OF
HASTINGS Operating as “QUINTE WEST
DAYCARE” and THE CITY OF QUINTE WEST

Defendants

REASONS FOR JUDGMENT

Gunsolus, J

Released: January _____, 2008.